Redesigning Rwanda's Health Insurance: Community and Institutional Interviews

Isomo Academy Action Research • The Detectives • Data Collection Tool (Google Form print version)

About this research

We are **The Detectives**, a student research team at **Isomo Academy**. Our action research is guided by one question: *How can we redesign Rwanda's health insurance system to serve every citizen and provide adequate financial protection?*

Rwanda has expanded health insurance coverage greatly, with about 90% of people insured according to our sources. But being insured does not always mean a family is protected from health costs. Our research looks at four connected issues:

1. **Ubudehe classification:** how households are placed in categories, whether this reflects their real lives, and what happens when a family's situation changes.
2. **Satisfaction with Mutuelle de Santé (CBHI) services:** how members experience care, costs, and treatment.
3. **Health financing:** how much money is allocated to health insurance and how it is managed.
4. **From money to services:** how that funding shapes the services people actually receive.

Who we are speaking with

CBHI members in the community; officials from **MINALOC** (Ubudehe classification criteria and process); and officials from **RSSB** (CBHI financing and complaints).

How your answers will be used

Your answers will be combined with those of other participants, analyzed, and used to propose an improved health insurance financing model, which we will present at Isomo Academy.

Your rights

Taking part is voluntary. You may skip any question or stop at any time with no consequences. We collect **no names, phone numbers, or emails**. Answers are stored securely, seen only by our team and teachers, and reported only as grouped results or anonymous quotes. This project is a student study and is not connected to any government decision about your insurance or category.

Time needed: about 15 minutes (community) or 30 to 45 minutes (institutions). **Research period:** October to November 2026. **Interviewers:** complete this form on behalf of the participant after the consent explanation.

* Indicates required question

Form settings: Collect email addresses OFF • Limit to 1 response OFF • Show progress bar ON • Confirmation message: *"Thank you. This interview has been recorded. The Detectives, Isomo Academy."*

SECTION S1: Start (everyone)

1. Interview ID *

Use U-001 (community), M-001 (MINALOC), R-001 (RSSB).

Logic: Response validation: regular expression `^[UMR]-\d{3}$`, error text: "Use U-001, M-001, or R-001".

2. Interviewer *

Choose one from the dropdown list.

- ◯ [Team member 1]
- ◯ [Team member 2]
- ◯ [Team member 3]
- ◯ [Team member 4]
- ◯ [Team member 5]

3. Date of interview *

Example: January 7, 2026

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4. Language of interview *

Mark only one oval.

- ◯ Kinyarwanda
- ◯ English
- ◯ French

5. Who is being interviewed? *

Mark only one oval.

- ◯ CBHI member (community)
- ◯ MINALOC
- ◯ RSSB

Logic: Go to section based on answer: CBHI member -> U1 • MINALOC -> M1 • RSSB -> R1.

SECTION U1: Consent (CBHI members)

Read aloud: "We are students from Isomo Academy studying how Mutuelle de Santé works for families, so we can suggest improvements. We are not from the government. There are no right or wrong answers. We will not write your name. You may skip any question or stop at any time. Your words may be used as an anonymous quote."

6. Is the participant 18 years or older? *

Mark only one oval.

- ◯ Yes
- ◯ No

Logic: Yes -> continue • No -> Submit form.

7. Did the participant agree to take part after hearing the explanation? *

Mark only one oval.

- ◯ Yes
- ◯ No

Logic: Yes -> continue • No -> Submit form.

8. May we use their words as an anonymous quote? *

Mark only one oval.

- ◯ Yes
- ◯ No

SECTION U2: Background

9. District *

Choose one from the dropdown list.

- ◯ [District 1]
- ◯ [District 2]
- ◯ [District 3]
- ◯ Other

10. Sector *

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11. Area type *

Mark only one oval.

- ◯ Rural
- ◯ Semi-urban
- ◯ Urban

12. Age group *

Mark only one oval.

- ◯ 18-25
- ◯ 26-40
- ◯ 41-60
- ◯ 60+

13. Gender *

Mark only one oval.

- ◯ Female
- ◯ Male
- ◯ Prefer not to say

14. Number of people in the household *

Number only.

Logic: Response validation: number between 1 and 30.

15. Main source of income *

Mark only one oval.

- ◯ Farming
- ◯ Casual work
- ◯ Small business
- ◯ Salaried work
- ◯ No regular income
- ◯ Other

16. Time to walk to the nearest health center *

Mark only one oval.

- ◯ Under 30 minutes
- ◯ 30-60 minutes
- ◯ 1-2 hours
- ◯ Over 2 hours

17. How long has the family been in Mutuelle? *

Mark only one oval.

- ◯ Under 1 year
- ◯ 1-3 years
- ◯ Over 3 years
- ◯ Not currently covered

SECTION U3: Ubudehe classification

18. Which Ubudehe category is the family in? *

Mark only one oval.

- ◯ Category 1
- ◯ Category 2
- ◯ Category 3
- ◯ Category 4
- ◯ Doesn't know
- ◯ Prefers not to say

19. How was the family placed in this category? (Tell it as a story: who came, what they asked or looked at)

20. Does the participant understand how the category was decided? *

Mark only one oval.

	1	2	3	4	5	
Not at all	◯	◯	◯	◯	◯	Fully

21. Does the category match the family's real life? *

Mark only one oval.

- ◯ Yes
- ◯ Somewhat
- ◯ No

22. Has the family's life changed in the last 3 years (new job, harvest, business, death)? *

Mark only one oval.

- ◯ Yes
- ◯ No

Logic: Yes -> next question • No -> skip to "Did the family try to correct the category?"

23. In what way did life change?

Check all that apply.

- ☐ Better income
- ☐ Lower income
- ☐ Job loss
- ☐ Illness
- ☐ Death in the family
- ☐ New business
- ☐ Other

24. Did the category change after that? *

Mark only one oval.

- ◯ Yes
- ◯ No
- ◯ Not applicable

25. Did the family try to correct the category? *

Mark only one oval.

- ◯ Yes
- ◯ No
- ◯ Didn't know it was possible
- ◯ Tried, but the system made it impossible

Logic: Yes or "Tried, but..." -> U3a • No or "Didn't know" -> U4.

SECTION U3a: Correction attempt (only if they tried)

After this section: Continue to U4.

26. What happened when they tried to correct the category? *

SECTION U4: Using Mutuelle services

27. Tell about the last visit to the health center with the card. What was it like? *

28. Overall, how satisfied is the participant with the services? *

Mark only one oval.

	1	2	3	4	5	
Very unhappy	◯	◯	◯	◯	◯	Very happy

29. Rate each part of the service (1 = very poor, 5 = very good) *

Mark only one oval per row.

	1	2	3	4	5
Waiting time	◯	◯	◯	◯	◯
Medicine available	◯	◯	◯	◯	◯
Respect from staff	◯	◯	◯	◯	◯
Clear explanation of costs	◯	◯	◯	◯	◯
Distance and transport	◯	◯	◯	◯	◯

30. Did the participant pay anything extra at that visit? *

Mark only one oval.

- ◯ Yes
- ◯ No

Logic: Yes -> U4a • No -> U4b.

SECTION U4a: Extra costs (only if Yes)

After this section: Continue to U4b.

31. What kind of extra cost? *

Check all that apply.

- ☐ Medicine
- ☐ Tests
- ☐ Transport
- ☐ Lost income from the day
- ☐ Other

32. About how much was paid extra (RWF)? *

Number only.

Logic: Response validation: number greater than or equal to 0.

SECTION U4b: Financial protection

33. How easy is the yearly contribution to pay? *

Mark only one oval.

	1	2	3	4	5	
Very hard	◯	◯	◯	◯	◯	Very easy

34. Has the family ever borrowed money or sold something to pay for health care? *

Mark only one oval.

- ◯ Yes
- ◯ No
- ◯ Prefers not to say

35. Has the family ever delayed or skipped care because of cost or distance? *

Mark only one oval.

- ◯ Yes
- ◯ No

Logic: Yes -> U4c • No -> U5.

SECTION U4c: Delayed or skipped care (only if Yes)

After this section: Continue to U5.

36. What was the main reason? *

Check all that apply.

- ☐ Cost of treatment
- ☐ Transport cost
- ☐ Distance
- ☐ Medicine unavailable
- ☐ Lost income
- ☐ Other

37. What did the family do instead? *

SECTION U5: Closing

After this section: SUBMIT FORM (do not continue to the MINALOC section).

38. What advice would the participant give to the people who run Mutuelle? *

39. Best quote (exact words, in Kinyarwanda and English)

40. Interviewer's notes (setting, mood, anything unusual)

41. How comfortable did the participant seem during the interview? *

Mark only one oval.

	1	2	3	4	5	
Uncomfortable	◯	◯	◯	◯	◯	Very comfortable

SECTION M1: Consent (MINALOC)

42. Interview mode *

Mark only one oval.

- ◯ In person
- ◯ Phone
- ◯ Online

43. Officer's role or department (name optional) *

_____

44. Position level *

Mark only one oval.

- ◯ Policy / leadership
- ◯ Technical / expert
- ◯ Local / field

45. Permission to record? *

Mark only one oval.

- ◯ Yes
- ◯ No, notes only

46. Consent given to take part? *

Mark only one oval.

- ◯ Yes
- ◯ No

Logic: Yes -> continue • No -> Submit form.

47. How may we cite this interview? *

Mark only one oval.

- ◯ By title and institution
- ◯ Anonymously
- ◯ Not for quotation

SECTION M2: Criteria and process

48. What criteria and indicators are used to place a household in an Ubudehe category? *

_____

_____

_____

49. Who collects the information, and how is it verified at community level? *

_____

_____

_____

50. How often is the classification updated? *

_____

SECTION M3: Transitions between economic levels (key section)

51. What does MINALOC do when households move from one economic level to another, whether improving or declining? *

Key question: required.

52. How does the system learn that a household's situation has changed? *

53. When a household moves to a higher category, what happens to its support? Is there gradual support so families do not fall back into poverty? *

54. How are households with unstable income handled (seasonal farmers, casual workers)? *

55. What happens to a household's health insurance support if its category changes in the middle of the year? *

56. Does a mechanism for handling transitions exist? (interviewer's judgment) *

Mark only one oval.

- ◯ Yes
- ◯ Partly
- ◯ No

SECTION M4: Appeals, reforms and links

57. How can a household appeal a category, and how long does it usually take? *

58. What are the most common errors or challenges in classification? *

59. How do the recent reforms change how households are classified? *

60. How does MINALOC work with RSSB so classification matches premiums and subsidies? *

61. What would make classification fairer and more accurate? *

SECTION M5: Data, documents and follow-up (MINALOC)

After this section: SUBMIT FORM.

62. Key figures mentioned (numbers, percentages, RWF amounts, dates)

63. Evidence strength of those figures *

Mark only one oval.

- ◯ Document provided
- ◯ Verbal only
- ◯ Estimate

64. Source type for the numbers *

Mark only one oval.

- ◯ Annual report
- ◯ Internal data
- ◯ Estimate
- ◯ Not stated

65. Did anything the official said contradict what community members told us?

66. Documents requested *

Check all that apply.

- ☐ Classification guidelines
- ☐ Reform document
- ☐ Statistics on category changes
- ☐ Other

67. Received? *

Mark only one oval.

- ◯ Yes
- ◯ No
- ◯ Pending

68. Link to the document (Drive link)

69. Follow-up needed? *

Mark only one oval.

- ◯ Yes
- ◯ No

70. Follow-up date

Example: January 7, 2026

71. Best quote

SECTION R1: Consent (RSSB)

72. Interview mode *

Mark only one oval.

- ◯ In person
- ◯ Phone
- ◯ Online

73. Officer's role or department (name optional) *

_____

74. Position level *

Mark only one oval.

- ◯ Policy / leadership
- ◯ Technical / expert
- ◯ Local / field

75. Permission to record? *

Mark only one oval.

- ◯ Yes
- ◯ No, notes only

76. Consent given to take part? *

Mark only one oval.

- ◯ Yes
- ◯ No

Logic: Yes -> continue • No -> Submit form.

77. How may we cite this interview? *

Mark only one oval.

- ◯ By title and institution
- ◯ Anonymously
- ◯ Not for quotation

SECTION R2: Money and allocation

78. How is the CBHI budget made up? What share comes from premiums, government, and partners? *

_____

_____

_____

79. How is the money divided between paying health facilities, administration, and other needs? *

_____

_____

_____

80. How are premiums and subsidies linked to each Ubudehe category? *

81. When funds run short, how does RSSB respond? *

SECTION R3: Services and payments

82. How long does it usually take to pay health facility claims? *

Mark only one oval.

- ◯ Under 30 days
- ◯ 30-60 days
- ◯ 60-90 days
- ◯ Over 90 days
- ◯ Not stated

83. What delays happen, and why? *

84. How does the money allocated to CBHI affect the services members receive (medicine, tests, referrals)? *

SECTION R4: Complaints (key section)

85. What is the most common problem in the complaints RSSB receives from CBHI members? *

Key question: required.

86. Top complaint type (interviewer's choice) *

Mark only one oval.

- ◯ Co-payments
- ◯ Medicine shortage
- ◯ Category error
- ◯ Claims
- ◯ Staff treatment
- ◯ Other

87. Numbers on complaints received (per year, by type)

88. What percentage of complaints are resolved, and in how long? *

89. How are complaints received, handled and resolved? What has changed because of them? *

SECTION R5: Transitions and sustainability

90. What happens in the insurance system when a member's Ubudehe category changes during the year? *

91. What are the biggest financial pressures on CBHI over the next five years? *

92. What one change would most improve financial protection for members? *

SECTION R6: Data, documents and follow-up (RSSB)

After this section: SUBMIT FORM.

93. Key figures mentioned (numbers, percentages, RWF amounts, dates)

94. Evidence strength of those figures *

Mark only one oval.

- ◯ Document provided
- ◯ Verbal only
- ◯ Estimate

95. Source type for the numbers *

Mark only one oval.

- ◯ Annual report
- ◯ Internal data
- ◯ Estimate
- ◯ Not stated

96. Did anything the official said contradict what community members told us?

97. Data requested *

Check all that apply.

- [ ] Revenue and expenditure
- [ ] Enrolment by category
- [ ] Complaints data
- [ ] Claims paid vs submitted
- [ ] Other

98. Received? *

Mark only one oval.

- ◯ Yes
- ◯ No
- ◯ Pending

99. Link to the document (Drive link)

100. Follow-up needed? *

Mark only one oval.

- ◯ Yes
- ◯ No

101. Follow-up date

Example: January 7, 2026

102. Best quote

Appendix A: How every question serves the research

Research focus	Form sections	How it will be analyzed
Ubudehe classification	U3, M2, M4	% who say the category matches real life; compare with official criteria
Satisfaction with CBHI	U4, U4b	Mean ratings by Ubudehe category, area type and distance
Money allocated	R2, R3, R6	Budget breakdown compared with costs users report
Money to services	U4a, U4b, U4c, R3	Extra costs, borrowing and skipped care by category
Transitions between levels	U3, M3, R5	What users experience vs what MINALOC and RSSB say exists
Complaints	R4, U4	Top complaint type vs what users say went wrong

Appendix B: Branching checklist (set these in Google Forms)

- Add all sections first, then name them S1, U1 to U5 (including U3a, U4a, U4b, U4c), M1 to M5 and R1 to R6.
- Interview type question in S1 -> Go to section based on answer: U1, M1, R1.
- Consent and age questions: No -> Submit form.
- Life changed = No -> skip the "In what way" and "Did the category change" questions.
- Tried to correct = Yes / Tried but impossible -> U3a; otherwise U4.
- Extra payment = Yes -> U4a; No -> U4b. After U4a -> U4b.
- Skipped care = Yes -> U4c; No -> U5. After U4c -> U5.
- After U5, M5 and R6 -> Submit form (the step most often forgotten).
- Make the key questions required: M3 transitions and R4 complaints.
- Test every path with fake responses, check the Sheet receives them, then delete the test rows.

Appendix C: Google Sheet setup

- Link the form: Responses -> Link to Sheets -> Create a new spreadsheet. Share with all team members as Editor.
- Keep the tab "Form Responses 1" untouched. Do all analysis in other tabs.
- Create one filtered tab per group, e.g. `=FILTER('Form Responses 1'!A:CZ, 'Form Responses 1'!E:E="MINALOC")` (adjust the column letter to where the interview-type answer sits).
- Add tabs: Data Log, Codebook, Theme Coding, Summary and Triangulation Matrix.
- Keep paper copies for villages with poor internet and enter data within 24 hours.